

رئيس
المجلس الطبي المتخصص

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Name: بشينه عبد العظيم عبد الوارث احمد National ID: 26604182501426	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

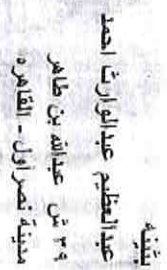
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bone scintigraphy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

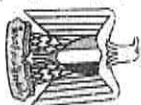


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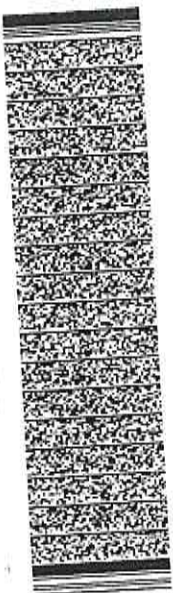
LB6983954

Handwritten symbols:

1


$$T^{\alpha\beta} = \frac{1}{2} \text{Tr}(\sigma^{\alpha} \sigma^{\beta})$$


انتى مسلمة متروجة
محتى الدين احمد تمام سيد
البطاقة سارية حتى : ٢٠٢٦/٠٣/١٤





Gustave Roussy International Egypt- Pathology Lab
جوستاف روسي انترناشيونال - مصر - مختبر الأمراض



Patient Name	بشينة عبدالعظيم عبدالوارث	Case No.	343632
Sex/Age	Female 60 Y	Patient No.	25101
Registered Date	22/7/2026 02:11:53 PM	Referred By Dr.	Out Patient
Validation Date	08/8/2026 03:44:23 PM	Branch	Gustave Roussy
Reporting Date	01/9/2026 10:59:04 AM	Pathology No.	

Clinical Data: Left breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 21/07/2026, at 12:30 pm.
Cold ischemia time is 20 seconds.

Gross Examination:

- Fragmented grey, white tissue cores, ranging in size between 0.2 cm & 0.8 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma.
- **Histologic grade:** 2 (differentiation 3, pleomorphism 2, mitosis 1).
- **Ductal carcinoma in situ:** Absent.
- **Microcalcification:** Present.
- **Angioinvasion:** Present.
- **Perineural invasion:** Absent.

Results:

* IPT (All controls showed appropriate reactivity):



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- PR: No nuclear reaction in tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 30%.

Conclusion:

Left breast mass, Tru-cut biopsy:

-Invasive duct carcinoma, Grade 2.

IPT:

- ER: Negative (score 0/8).
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****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

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Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the



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Signature

Dr. Walaa Ibrahim Ghonaim, MD.



Presidential Program for Women's Health

Name: بشينه عبدالعظيم عبدالوارث

Date: Saturday, July 18, 2026

Bilateral Full Field Digital Mammography and Ultrasound

Clinical Summary:

Left breast palpable lump

Technique:

MLO and CC views

High Resolution Ultrasound

Detailed Findings:

Left breast:

- (ACR B).
- The left breast shows irregular ill-defined spiculated dense mass lesion associated with architectural distortion, nipple retraction and mild overlying skin thickness
- Ultrasound demonstrated
- The left breast shows one ill-defined hypoechoic irregular shaped mass lesions with spiculated margins *seem to be interconnected, they measures globally about 3x5 (Corresponding to palpable Lump) located at 6 o'clock birads 5*

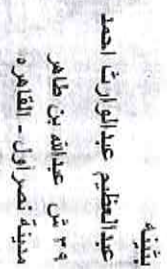
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- (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound demonstrated
- No solid or cystic lesions
- Normal skin/nipple/areolar complex.
- Preserved axillary node.

Conclusion:

- Left circumscribed speculated radiopaque intra mammary lesion suspicious for biopsy

Best regards,
Dr.Dr.fatma atef



Y 77. 318 Y 0. 13 Y 7

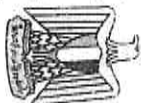
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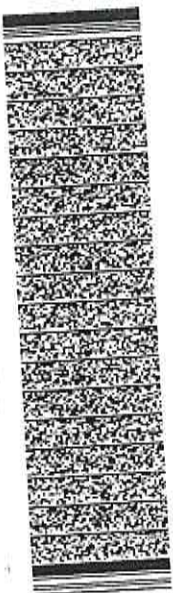


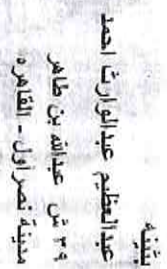



1


$$r_{\text{TC}} = 1.87 \times 10^{-1} \text{ cm}^2 \text{ s}^{-1}$$


انتى مسلمة
محتى الدين احمد تمام سيد
متروجه
البطاقة سارية حتى : ٢٠٣٦/٠٣/١٤





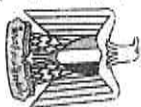
Y 77. 318 Y 0. 13 Y 7

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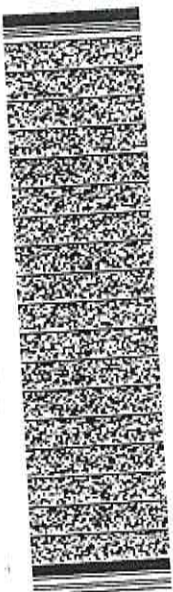
Handwritten symbols:



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$$r_{\text{TS}} = 1.87 \times 10^{-10} \text{ m}$$


انتش
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محمي الدين احمد تمام سيد
البطاينة سارية حتى : ٢٠٣٢/٠٣/١٤



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خريطة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - (1503) في - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Nasser El Qadema - Cairo, Egypt

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - معمل الباثولوجي - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt



Presidential Program for Women's Health

Name: بثينة عبدالعليم عبدالوارث

Date: Saturday, July 18, 2026

Bilateral Full Field Digital Mammography and Ultrasound

Clinical Summary:

Left breast palpable lump

Technique:

MLO and CC views

High Resolution Ultrasound

Detailed Findings:

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- No solid or cystic lesions
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Conclusion:

- Left circumscribed speculated radiopaque intra mammary lesion suspicious for biopsy

Best regards,

Dr.Dr.fatma atef

تعرفني على كيفية إجراء الفحص الذاتي للثدي

الخطوة الأولى (أمام المرأة)

- انظري إلى الثديين في المرأة مع وضع اليدين في وضع مستقيم على الجانبين ثم وضع اليدين على الوسط.
- تأخدي بالنظر إلى شكل ولون وحجم الثديين، وأنه هو المعتاد بالنسبة لكي دون تغيير أو دون اختلاف بين الثديين.
- لاحظي وجود أي من العلامات التالية بأي من الثديين :

بروز الأوردة تغير في الشكل أو الحجم حدوث تجوف طفح

تجاعيد بالجلد تحول الجلد لما يشبه قشر البرتقال الإحمرار تراجع أو تغيير في اتجاه الحلمة تقرحات بالجلد أو الحلمة

الخطوة الثانية (أمام المرأة)

- ارفعي يديكي إلى أعلى وتأخدي من عدم وجود أي من التغيرات السابقة

الخطوة الثالثة (أمام المرأة)

- أثناء رفع يديكي إلى أعلى، لاحظي خروج أي إفرازات غير طبيعية أو دم

الخطوة الرابعة (على السرير)

- استلقي على السرير، إستخدمي اليد اليسرى لفحص الثدي الأيمن واليد اليمنى لفحص الثدي الأيسر.
- باستخدام أطراف أصابعك و من خلال حركة دائرية، قومي بفحص الثدي (الجلد والأنسجة).
- تأخدي من فحص الثدي بالكامل من أعلى إلى أسفل، ومن جانب إلى جانب - من عظمة الترقوة إلى أعلى بطنك، ومن الإبط إلى منتصف الصدر.
- حتى تتأخدي من فحص الثدي بالكامل، يمكنك أن تبدأي من الحلمة ثم تتحركي في دوائر أكبر وأكبر حتى تصلي إلى المنطقة الخارجية للثدي.
- يمكنك أيضاً تحريك أصابعك لأعلى ولأسفل رأسياً.
- لاحظي الإحساس بأي من العلامات التالية:

الشعور بأورام بالإبط الشعور بسمك عام

ورم محسوس

ألم بالثدي

الخطوة الخامسة (أثناء الإستحمام)

- أعيدي الفحص السابق بنفس الطريقة المذكورة أثناء الإستحمام والجلد مبلل.

أكملت توقيت لتقومي بالفحص الذاتي للثدي بعد آخر يوم في الدورة الشهرية أو في حالة انقطاع الدورة الشهرية صحتي يوم ثابت شهرياً لإجراء الفحص

إذا لاحظت أي من العلامات السابقة فمعي إلى زيارة طبيبة

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- Perineural invasion: Absent.

Results:

* IPT (All controls showed appropriate reactivity):

Patient's Name: Butaina Abdel Azim Abd al-Warith Ahmed
Date: 2026-09-12
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- Left breast mass lesion is seen measuring 38 x 35 mm with left axillary enlarged lymph nodes largest measuring 34 x 17 mm for sono-mammography.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ Left breast mass with axillary lymphadenopathy for sono-mammography.
- ❖ No pulmonary or hepatic metastatic lesions.

Dr.Sherif Wagdy

Patient Name : **بنيته عبدالمظيم عبدالوارث احمد**
 Patient Info. : **Female 60Y**
 Department : **Out Patient**
 Profile NO. : **26604182501426 / 3363-2026 / 13625**

Patient Id : **25101**
 VisitId : **384011**
 Collection Date : **12/09/2026 11:57:45**
 Reporting Date : **12/09/2026 12:40:27**

Lipid Profile

Test	Result	Unit	Reference Range
Lipids Profile			
Total Cholesterol	: H 270	mg/dL	Desirable < 200 Borderline 200-239 Higher risk > 240
Triglycerides	: 99	mg/dL	0 - 200
HDL Cholesterol	: 51	mg/dL	Low HDL < 40 High HDL >=60
LDL Cholesterol	: 199	mg/dL	Desirable : <100 mg/dl Above Desirable : 100-129 mg/dl Borderline : 130 - 159 mg/dl High : 160 - 189 mg/dl very High : >= 190 mg/dl Less Than 32 Up to 5.5
VLDL Cholesterol	: 20	mg/dL	1/2 Average : < 3.9
Risk ratio I	: 5.3		Average : < 5.0
Risk ratio II	: 3.9		2 Average : < 9.6 3 Average : < 23.4

Signature

Dr. Reaam Abdelmonem

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Patient Info. : Female 60Y
Department : Out Patient
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Patient Id : 25101
VisitId : 384011
Collection Date : 12/09/2026 11:56:09
Reporting Date : 13/09/2026 15:31:49

Clinical Chemistry Report

Test	Result	Unit	Reference Range
Diabetes Profiles			
Hb A1-C	: H 7.70	%	Normal : 4.5 -5.7 prediabetic : 5.8 - 6.4 Diabetic : >= 6.5 Good Control : 6 - 7 % Fair Control : 7 - 8 % Poor Control : > 8 %
Kidney Function			
Serum Creatinine	: 0.85	mg/dL	0.58 - 1.3

Comment:

The test was performed at an external reference Lab (Nile Lab)
Signature by Dr. Ashraf Deiaa

Signature

Dr. Reaam Abdelmonem

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Reporting Date	01/9/2026 10:59:04 AM	Pathology No.	

- ER: No nuclear reaction in tumor cells (score 0/8).
- PR: No nuclear reaction in tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 30%.

Conclusion:

Left breast mass, Tru-cut biopsy:

-Invasive duct carcinoma, Grade 2.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67%: 30%.

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

** Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the

Patient Name	بنته عبدالعظيم عبدالوالت	Case No.	343632
Sex/Age	Female 60 Y	Patient No.	25101
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invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within <10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense >10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaini, MD.